HEALTHTRACK

Master Product Blueprint + PRD + Investor Business Case

HealthTrack 1.0 launch architecture | HealthTrack 2.0 expansion

Core product philosophy: Personal wellness model
Know your normal. Notice what matters. Do ONE thing. Repeat what works.

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Working strategy document - assumptions to be validated through beta and market testing



The new HealthTrack

From a health-data dashboard → to a Personal Wellness Guidance System

The central promise should no longer be:

> "We bring all your health data together."

That is becoming commodity functionality.

Instead:

> **HealthTrack understands what is changing in your body, identifies what may be influencing it, and helps you improve your wellness—one behavior at a time.**

That distinction is subtle, but strategically enormous.

Your sentence captures the philosophy beautifully:

> **"Detection without behavior change is just earlier anxiety."**

1. Make Wellness the center—not a fifth pillar

Instead of:
Sleep | Recovery | Activity | Nutrition | Wellness

HealthTrack should become: WELLNESS
supported by four continuous behavioral systems

Sleep → Wellness ← Nutrition
Recovery → Wellness ← Fitness & Activity

And underneath everything:
Labs + Wearables + Lifestyle + User Context

The product is therefore not asking:
“How was your sleep?”

It is asking:
“What is your body telling us today—and what is the most useful thing you can do about it?”

That should become the conceptual center of HealthTrack.

2. The real differentiator: ONE Behavior

This is where I think HealthTrack can become genuinely interesting.

- Oura gives insights.
- WHOOP gives recommendations and Action Plans.
- Apple aggregates enormous amounts of health data.
- Function gives deep biomarker information.

HealthTrack should specialize in something slightly different:
Insight → Decision → Behavior → Response

Every day the AI Health Coach determines:
“What is the ONE behavior most likely to improve Ram's wellness today?”

That might be:
Today's ONE
Take a 20-minute walk within an hour after dinner.
Then HealthTrack observes what happens afterward.
Did sleep improve?
Did resting HR change?
Did HRV change?
Did glucose data improve, if available?
Did mood/energy improve?
Did the user actually do it?
Over weeks HealthTrack begins learning:
What works for this individual?
That becomes much more interesting than generic AI recommendations.

3. HealthTrack should develop a Personal Response Model

This is where I see your long-term moat.

Most health applications are essentially:
Population knowledge → User data → Recommendation

HealthTrack should evolve toward:
Population knowledge + User data + User's historical response → Personalized intervention

Suppose HealthTrack learns over six months:
Ram's recovery improves significantly when sleep exceeds 7h 15m.
And:
Late meals correlate with lower overnight recovery.
And:
Moderate evening walking correlates with better sleep.
And a lab report shows:
HbA1c trending upward.

Now the Health Coach doesn't simply say:
“Exercise helps glucose control.”
Instead:
Your data suggests something more specific.
On days when you walk 15–25 minutes after dinner, your overnight recovery tends to be better.
Because your recent HbA1c has also moved upward, let's make post-dinner walking your focus for the next 14 days.

That's personalization.
And after 14 days:

Did it work?
Yes / No / Uncertain

HealthTrack learns.
That creates a closed-loop system.

4. Adaptive Behavior Intelligence

“We combine wearables + labs + AI” is no longer a differentiation strategy.

HealthTrack needs another layer.

I would make that layer: **Adaptive Behavior Intelligence**

Not just:

What does your data mean?

But:

What should YOU do next—and did it actually work?

That distinction needs to permeate the product.

5. Personal Baseline Intelligence

Most healthcare operates against population ranges.

For example:

HRV: population comparison
Resting HR: population comparison
A1c: reference range
Vitamin D: reference range

HealthTrack adds another question:

“What is normal for YOU?”

Imagine HealthTrack learning:

Your baseline

Sleep: 7h 18m
RHR: 61
HRV: 47
Activity: 7,800 steps

Energy: 7.4/10
Stress: 4.1/10

Then something changes.

Sleep ↓ 12%
HRV ↓ 17%
RHR ↑ 6 bpm
Activity ↓ 22%
Energy ↓
Stress ↑

No single number necessarily means anything medically.

But collectively: You're moving away from your baseline.

HealthTrack could say:

> *Your wellness pattern has shifted over the last four days.*
>
> *Sleep consistency and recovery have both moved away from your typical range.*
>
> *Nothing here necessarily indicates a medical problem. Your recent patterns suggest that prioritizing recovery may be useful.*
>
> *Today's ONE: Aim for your normal bedtime tonight.*

That's exactly the territory HealthTrack should occupy.

Executive Summary

HealthTrack is positioned as a personal wellness guidance system rather than another health-data dashboard. The platform integrates wearable signals, parsed laboratory reports, nutrition, activity, recovery, sleep, self-reported context and AI-assisted explanation. Its differentiator is Personal Baseline Intelligence: learning what is normal for an individual, identifying meaningful deviations, prioritizing one low-risk behavior that matters today, and learning which behaviors are repeatedly associated with better outcomes for that person.

HealthTrack's strategic wedge

Wearables measure continuously. Labs provide deeper episodic context. HealthTrack sits between them as the behavior-intelligence layer:
My Wellness Status -> Today's ONE -> Repeat ONE -> learn what works for me.

Product North Star

- Primary launch customer: U.S. direct-to-consumer wellness user.
- Future surfaces: corporate wellness, schools/athletic programs, and provider-authorized longitudinal views.
- HealthTrack 1.0: hardware-neutral data aggregation, parsed labs, Personal Baseline Intelligence, My Wellness Status, Today's ONE, Repeat ONE and AI Health Coach.
- HealthTrack 2.0: biomarker testing partnerships/offers, deeper longitudinal biomarker intelligence, behavior experiments, organizational dashboards and provider workflows.

1. Product Principles

Principle	Implication
Wellness is the outcome	Sleep, Recovery, Nutrition and Fitness & Activity are supporting lenses, not competing destinations.
Personal first	The strongest comparison is the user's own longitudinal baseline; peer and general references add context.

One action, not ten	Detection without behavior change can create anxiety. HealthTrack prioritizes one feasible behavior.
Learn from response	Recommendations become more personal as adherence and subsequent signals accumulate.
Explain, don't diagnose	HealthTrack 1.0 stays focused on general wellness; AI explains deterministic signals and does not independently diagnose.
Hardware neutral	The intelligence layer should work across supported wearable ecosystems rather than require proprietary hardware.

2. Users and Jobs-to-be-Done

Segment	Primary job	1.0 / Later
DTC consumer	Understand my current wellness pattern and know the most useful behavior to focus on today.	Launch
Corporate wellness	Help employees build healthier behaviors while giving employers appropriately aggregated program insights.	Later dashboard
Schools / athletics	Support athlete wellness, recovery and behavior routines with consent and role-based visibility.	Later dashboard
Providers	See patient-authorized longitudinal context between visits without	Later dashboard

	replacing clinical judgment.	

3. HealthTrack 1.0 Information Architecture

Primary navigation: Today | Trends | Progress | Profile. AI Health Coach is a persistent contextual action from Today and Trends rather than a fifth analytics tab.

Surface	Question answered	Core content
Today	What matters now?	My Wellness Status; Today's ONE; Repeat ONE; compact pillar states; reason/explanation.
Trends	What is changing?	Deviation from personal baseline; pillar trends; labs; cross-signal associations; confidence.
Progress	What appears to work for me?	Behavior history; adherence; observed associations; Personal Behavior Library; weekly/monthly reflection.
Profile	Who am I and what data may HealthTrack use?	Goals; demographics; preferences; devices; lab uploads; permissions; privacy; subscription.
AI Health Coach	Why, and what next?	Context-aware explanations, questions about status/trends/labs, and safe behavior guidance.

4. Today Screen Specification

- Header: greeting, date, AI Health Coach access.
- My Wellness Status: state-first presentation (recommended states: Thriving, Stable, Shifting, Needs Attention), with 7-day direction and confidence.
- Supporting lenses: Sleep, Recovery, Nutrition, Fitness & Activity, Labs. Each shows direction relative to personal baseline; tap for explanation.
- Today's ONE: the single highest-priority feasible low-risk behavior for today, with a short 'Why this?' explanation and adherence control.
- Repeat ONE: a previously attempted behavior with sufficient positive personal evidence to be worth repeating.
- Optional deeper context remains one level below; Today must not become a dense dashboard.

5. Personal Baseline Intelligence

HealthTrack should distinguish three reference layers and never imply that a population comparison is the same thing as an individual's healthy baseline.

Reference layer	Purpose	Display priority
My Baseline	Rolling, quality-controlled personal distribution and trajectory for each supported metric.	Primary
Peer Reference	Age/sex and only scientifically validated demographic/contextual factors where appropriate.	Secondary
General Reference	Established broad population or laboratory reference context.	Secondary

Ethnicity should not be mechanically applied to every metric. It should be used only where credible evidence or validated reference standards support meaningful interpretation.

6. My Wellness Status - Algorithmic PRD

The core status engine should be deterministic, versioned and auditable. The LLM explains the result; it does not invent or calculate the underlying state.

Input dimension	Examples
Signal deviation	Distance from personal baseline, direction and magnitude.
Persistence	How long the deviation has lasted.
Cross-signal agreement	Whether multiple independent signals move together.
Data quality	Device coverage, missingness, recency, lab provenance.
Context	Travel, illness flag, menstrual context where provided, unusual schedule, self-report.
Safety rules	Suppress/redirect recommendations when a signal warrants clinical evaluation or is outside general-wellness scope.

Proposed internal status logic

For metric i, compute a normalized personal deviation d_i using a robust rolling baseline (for example median and median absolute deviation or another validated method), then apply reliability r_i, persistence p_i and domain weight w_i. Domain scores aggregate only eligible metrics. A separate rules engine maps the internal composite and persistence pattern to consumer-facing states. Thresholds must be calibrated during beta; the values are not yet clinically validated.

Illustrative internal form: DomainDeviation = sum(w_i * r_i * p_i * f(d_i)) / sum(w_i * r_i). The consumer sees a state and explanation, not a pseudo-clinical 0-100 health score.

- Baseline confidence: Building -> Moderate -> Established, based on sufficient valid days and source consistency.
- Cold start: use user profile + peer/general context conservatively while clearly labeling that HealthTrack is still learning the user.

- Lab trends: use laboratory reference intervals and longitudinal change; never substitute HealthTrack's wellness interpretation for clinician diagnosis.

7. Today's ONE and Repeat ONE

Engine	Purpose	Candidate ranking
Today's ONE	Choose the one behavior most relevant to the current wellness pattern.	Relevance x evidence x expected personal benefit x feasibility x confidence x preference, penalized for burden/risk/uncertainty.
Repeat ONE	Surface a behavior with enough repeated personal observations to justify another trial.	Adherence history x consistency of associated response x recency x confidence x feasibility.

- Behavior candidates come from a curated, evidence-informed low-risk wellness library.
- LLM may personalize wording and explanation, but may not create an unsafe intervention outside policy.
- Observed response is labeled association, not causation, unless a future validated experimental framework supports stronger claims.
- Users can decline, replace, snooze or explain why a behavior is not feasible; this feedback updates personalization.

8. Trends, Progress and AI Coach

Trends

Start with 'What's Changing?' and rank meaningful deviations from personal baseline. Allow drill-down to Sleep, Recovery, Nutrition, Fitness & Activity and Labs. Cross-pillar insights should state confidence and use association language.

Progress

Progress is the learning surface: behaviors attempted, adherence, observed response, weekly reflections and a Personal Behavior Library. Its core promise is not 'you logged more data' but 'we are learning what appears to work for you.'

AI Health Coach behavior contract

- Context-aware: can reference current status, baseline, recent trends, labs, goals, prior ONEs and adherence.
- Priority-preserving: defaults to the highest-priority issue instead of generating a long list.
- Transparent: distinguishes measured data, inferred relationships, general evidence and uncertainty.
- Escalation-aware: identifies when a user should seek professional/urgent care rather than continuing wellness coaching.
- Non-diagnostic in 1.0: avoids claims that HealthTrack detects, prevents or treats disease.
- Auditable: store prompt version, model, context summary, output, safety flags and user feedback.

9. Onboarding

1. Welcome: HealthTrack learns your normal and helps you improve one behavior at a time.

2. Goals and priorities.

3. Account + consent + privacy choices.

4. Personal profile: age, sex, height, weight and optional relevant context.

5. Dietary preferences, restrictions, allergies, medications/supplements with skip options.

6. Typical day: sleep, activity, meals, work/school schedule and stress context.

7. Connect data: supported Junction/wearable sources and required permissions.

8. Upload recent lab report (optional; can defer).

9. Nutrition and recovery baseline questions.

10. Notifications and coaching preference.

11. Learning Your Baseline: explain confidence-building period and what the user will see on Today.

10. Technical Architecture

Design as a multi-tenant, role-aware platform even though only the DTC dashboard launches first.

Layer	Recommended responsibility
Client	React/mobile-responsive app initially; native/mobile shell later if required.
Identity & data	Supabase Auth, PostgreSQL, Storage, Row Level Security, audit logs.
Integration	Junction for supported wearable/health data; lab PDF ingestion/parsing; manual check-ins.
Intelligence services	Baseline service; Wellness Status service; behavior-ranking service; association/response service.
AI orchestration	Provider-agnostic model gateway supporting OpenAI/Gemini/other approved models; structured outputs and safety policies.
API	Versioned server-side API; never expose privileged keys or core algorithms in client.
Deployment	GitHub source control; Vercel for web demo/launch where appropriate; CI/CD and environment separation.
Analytics	Product analytics, recommendation acceptance, adherence, retention, data coverage and safety-event metrics.

11. Core Data Model

users / profiles, organizations / memberships / roles, consents / permissions, data_sources / devices, raw_observations / normalized_metrics, lab_reports / lab_results, baseline_snapshots / baseline_confidence, wellness_status_snapshots, behavior_library, recommendations, behavior_commitments / adherence_events, outcome_windows / behavior_associations, coach_sessions / safety_events, subscriptions / entitlements.

Organization and role tables should be present early so future corporate, athletics and provider dashboards can be added without re-keying ownership of personal health data. Individual-level sharing must be permissioned; employer views should default to appropriately aggregated program insights.

12. Nonfunctional Requirements

- Privacy/security by design: least privilege, encryption in transit/at rest, RLS, auditability, secrets management, environment separation.
- Data provenance: retain source, timestamp, device/lab origin, unit, normalization version and confidence.
- Explainability: every Wellness Status driver and ONE recommendation must be traceable to inputs/rules.
- Reliability: graceful handling of missing wearable days, duplicate data, timezone changes and disconnected sources.
- Accessibility: WCAG-oriented mobile UI, readable typography, color not used as the sole status cue.
- Regulatory boundary: general-wellness language for 1.0; formal regulatory review before adding disease claims or regulated functions.

13. MVP Acceptance Criteria

- User can create/sign in to account and complete onboarding.
- User can connect at least one supported health-data source and see normalized data.
- User can upload a lab report and review parsed results with provenance.
- System builds and displays baseline confidence and personal baseline for eligible metrics.

- Today displays My Wellness Status with explainable drivers.
- System produces exactly one Today's ONE when eligible, records acceptance/decline and adherence.
- Repeat ONE appears only after minimum evidence rules are met.
- Trends shows personal-baseline deviations and source confidence.
- Progress shows behavior attempts and learned associations.
- AI Coach can explain current status and recommendations without bypassing safety rules.
- All sensitive data access is role/permission controlled and auditable.

14. Product Analytics / KPIs

Category	Launch metrics
Activation	Onboarding completion; wearable connection; lab upload; first ONE accepted.
Engagement	DAU/WAU; Today opens; Coach use; trend drill-down; ONE acceptance.
Behavior	ONE completion; Repeat ONE rate; weekly adherence; replacement/decline reasons.
Learning	% users with Established baseline; % with qualified behavior associations; time to first Repeat ONE.
Retention	D30/D90 paid retention; monthly churn; annual renewal.
Business	Trial-to-paid; ARPU; CAC; gross margin; LTV/CAC; payback period.
Safety/quality	Escalations; recommendation suppressions; parsing corrections; user-reported incorrect insights.